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02.10 Introduction

Assessment is a method of determining an applicant's current long-term care needs by using a comprehensive standard instrument. Information obtained during the assessment process should be adequate for the **SCDHHS worker** to make a level of care decision and to initiate service planning for discussion with the applicant and/or Authorized Representative.

An assessment form must be completed for any person referred to CLTC who meets the intake criteria unless the applicant is terminated prior to assessment completion.

02.20 **Assessment Policy and Procedure**

02.21 **Consent Form**

South Carolina Long Term Care Assessments submitted to the local CLTC Area Office from outside entities must include a signed Consent Form, DHHS Form 121, signed by the applicant or Authorized Representative. The purpose of this form is to ensure that the applicant, whenever possible, is involved in planning long term care needs and to obtain consent from the applicant for CLTC to share information with involved service providers.

Note: For nursing facility cases, continue to accept the previous hard copy Consent Form until the presently used Phoenix form may be printed.

If a case is closed for any reason and later reopened, a new Consent Form must be obtained. An exception to this policy is made for re- certifications. (See Chapter 8 Nursing Home.)

The Consent Form may be signed by the Authorized Representative only when the applicant is not mentally or physically able to sign the form, or at the applicant's request. It must be signed before the **SCDHHS worker** can take any action on the case.

If an applicant signs by making a mark, the form must be witnessed by the person receiving the consent and one other person.

It is appropriate to accept verbal consent when all other options are exhausted or when the assessment is being completed by phone. The verbal consent must be signed by the person receiving the consent and one other witness.

The **SCDHHS worker** updates the Consent obtained date field of Participant Information in Phoenix each time a new Consent is obtained.

02.22 Participant Service Choice

A written Participant Service Choice Form must be secured from the applicant to ensure that the applicant is involved in long term care planning. This location choice will remain in effect until such time as the applicant changes the location choice or until the case is terminated. If the applicant lacks the physical or mental ability required to make a written choice regarding care or at the request of the applicant, the Authorized Representative may sign the Participant Service Choice, DHHS Form 164. In these cases, if the Authorized Representative is not available, efforts should be made to make a phone contact with the Authorized Representative to discuss location choice and document the discussion in the Comment Section of the form.

The absence of a signature from the applicant or Authorized Representative must be documented in the Comment Section of the Participant Service Choice, DHHS Form 164. For waiver application, the **SCDHHS worker** or Contract Case Manager must obtain the signature of the participant and/or Authorized Representative on the Participant Service Choice Form in Phoenix during the Initial Visit.

The Participant Service Choice SCDHHS Form 164 is required for all CLTC waivers. The Participant Service Choice SCDHHS Form 164 is not required for participants entering a Nursing Home from the community or from a hospital or in instances of a Nursing Home conversion.

An individual has the right to assume risk, commensurate with that person's ability and willingness to assume responsibility for the consequences of that risk. This, of course, does not abrogate a State's statutory duty to ensure the health and welfare of individuals served under the Home and Community-Based Services waivers. Staff should exercise good professional judgment in dealing with cases where the applicant's health, safety, and welfare are questionable. Cases should be discussed with designated Lead Team and Area Administrator. The CLTC Upstate Director/Low Country Director and Central Office Quality Assurance should be consulted. Legal counsel may also be needed. Documentation is crucial in these cases. Referrals must be made to other agencies (DMH, DSS, etc.) as appropriate and/or as required by State law.

2.22.01 Appointing an Authorized Representative

An Authorized Representative is an individual or organization appointed by the waiver participant to act on the participant's behalf. An Authorized Representative has authority to file and represent the beneficiary in appeals, receive information regarding the participant's medical condition, and provide necessary information on how the participant performs activities of daily living. The Authorized Representative can also communicate regarding the participant's service needs and choice of provider.

Appointing an Authorized Representative for Long-Term Care Services – Form 1282 CLTC

- Name, Address and Phone number must be completed for each Authorized Representative.
- A separate Form 1282 CLTC must be completed in its entirety for each Authorized Representative to be appointed. (Maximum of 3)
- Signature: The form must be signed by the participant or their legal representative.
 - Verbal Authorization will be accepted if the Verbal Authorization section of the form is completed in its

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- entirety. If verbal authorization is received, physical signature will not be required.
 - If the participant is signing with an “x”, two (2) witnesses of the participant’s signature are required.
 - Note: Case Manager can be one of the witnesses as long as they witnessed the participant sign the form and there is an additional witness present.
 - Please note that the Authorized Representative cannot serve as a witness.
 - Mental Incompetence: If a participant is mentally incompetent (does not understand), only a legal representative can appoint another individual/entity to act as the participant’s Authorized Representative.

Note: If the participant has an intellectual disability or diagnosis dementia but has not been legally declared incompetent, they can appoint their Authorized Representative and sign their own consents.

- Physical Incapacity: If a participant is physically incapacitated (cannot communicate verbally, by gesture or other means), only a legal representative can appoint another individual/entity to act as the participant’s Authorized Representative.
- Legal Representative: A legal representative is someone that is appointed by the participant through a Power of Attorney OR appointed by a Court to act on behalf of the participant. The legal representative must provide a copy of the document evidencing their authority to act on the participant’s behalf in order for their authority to appoint an Authorized Representative to be recognized.
- A Form 1282 CLTC is binding until revoked or until the waiver case is closed.

Appointment of an Authorized Representative is not required, a waiver participant has the right to represent themselves. Should the waiver participant waive their right to appoint an Authorized Representative, the appropriate box on the Form 1282 CLTC should be selected.

- If a Form 1282 CLTC was not obtained during the Intake referral process, the SCDHHS area office nurse or social worker will perform a phone assessment and complete the Form 1282 CLTC during the assessment indicating an Authorized Representative or that the participant has declined to appoint an Authorized Representative.
- A Form 1282 CLTC can be mailed to the waiver participant if

desired.

For existing participants:

- The Case Manager will check the Authorized Representative Tab under forms for all participants on their caseloads to see if a Form 1282 CLTC has been completed. The Case Manager will obtain the completed Form 1282 CLTC from each participant who does not already have the form completed.
- A completed Form 1282 CLTC is required to be in each file for existing participants by the beginning of the next quarter following implementation of this policy.

Quarterly Contact: These questions must be addressed by the Case manager during the quarterly contact phone call:

- Does the participant have a signed 1282 CLTC on file?
- Does the participant need to make a change to Form 1282 CLTC? If so, has the Revocation form been received by the participant?
- If applicable, Does the participant have a signed Form 121 on file?

A waiver participant can appoint up to three (3) Authorized Representatives at a time. Should the waiver participant choose to appoint more than one Authorized Representative, they all share equal power to act on the participant's behalf. There is no ranking or system of succession among the appointed Authorized Representatives.

Information regarding a participant (including application status) can only be **released** to an appointed Authorized Representative. While SCDHHS cannot release information, we can accept information from sources other than the Authorized Representative.

Documenting Authorized Representatives

The Authorized Representative's name is entered in Phoenix in the Caregiver Supports dashboard. The name of the Authorized Representative should be saved when uploading the completed Form 1282 CLTC in Phoenix under the Authorized Representative Tab in Scans.

Confirming Authority of Authorized Representative

The SCDHHS worker or Case Manager must review the Caregiver Supports Section to confirm an Authorized Representative Form is on file before releasing any information on the participant.

02.22.02 FORM 121– Consent Form

Form 121 gives SCDHHS area office staff/contract Case Managers:

- authorization to receive information from providers and
- authorization to disclose information to providers and outside agencies regarding waiver participants.

By signing this form, the participant authorizes social service professionals, doctors, nurses, and other providers to release information to SCDHHS area staff/Case managers regarding the participant's diagnosis and recommended treatment.

This form also authorizes SCDHHS to release information to providers when referring the participant for care or treatment.

This form also authorizes SCDHHS to inform Alzheimer's disease and Related Disorders Registry of certain related diagnoses.

This form also authorizes SCDHHS to release information to certain law enforcement agencies when there is a reasonable belief that the participant is a victim of abuse, neglect or exploitation.

However, the participant may also indicate organizations, agencies or persons to whom their information should NOT be shared.

For Adult Protective Services Cases (APS), if the SCDHHS area staff/contract Case manager is making the APS report for suspected abuse, SCDHHS can disclose information without a signed Form 121 LTL. If SCDHHS area office staff did NOT make the APS referral and information is being requested, then Form 121 LTL is required.

SCDHHS area staff/contract Case Managers are responsible for sending Form 121 LTL to outside agencies requesting information about a waiver participant. Form 121 LTL is mailed to outside agencies upon request.

A waiver participant may revoke or change this consent at any time by completing the Revocation of Authorization to Disclose Health Information.

The Revocation of Authorization MUST include:

- Name of the waiver participant
- Name of the person who signed the authorization
- date of signature
- Effective date of the withdrawal/revocation
- Signatures and date of the waiver participant or Legal Representative
 - If signed by a Legal Representative, documentation or their authority must be attached (i.e., POA)

Upon receipt of a Revocation of Authorization form, the SCDHHS area staff/Case Manager must 1) submit the form for legal review through Service Manager and 2) Scan the form to the Phoenix record.

If an appointed Authorized Representative chooses to no longer act in that capacity, the Authorized Representative must contact the agency to inform them of their decision to no longer act as the Authorized Representative. The Case Manager should notify the participant that the Authorized Representative no longer wishes to act as their Authorized Representative.

The following forms can be found in Phoenix Help and must be uploaded to the participant's Phoenix record if applicable:

- Appointing an Authorized Representative Form 1282 LTL
- Authorization to Disclose Health Information Form 921
- Revocation of Authorization to Disclose Health Information
- Revocation of Appointing an Authorized Representative Form

Note: If a SCDHHS Worker is contacted by a non-authorized representative, information can be obtained/received from this person, but no information can be disclosed/given.

02.23 Individuals Who May Complete Pre-Admission Screening Assessment

Individuals who may complete an assessment are as follows:

1. SCDHHS worker.

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2. Any person who is professionally involved in the applicant's case and is a nurse, social worker, social services worker, and/or physician.

No portion of the assessment can be completed by any person who is related to the applicant.

02.24 How to Determine Who Will Complete the Assessment

The location of the applicant at the time of assessment may determine who will complete the assessment.

An applicant located in a Medicaid certified long-term care facility will be assessed by the staff of that facility. When an applicant is in a non-Medicaid certified facility, the assessor will be determined by a Memorandum of Agreement (MOA) between the SCDHHS AreaOffice and the Nursing Home Administrator.

If no agency is currently involved with the applicant or if that agency is unable to complete the assessment, a **SCDHHS worker** will complete the assessment.

An initial waiver assessment may be completed on a hospitalized applicant by a **SCDHHS worker** or by hospital staff trained to complete the CLTC assessment tool.

02.30 Timeliness Standards for Completion of an Assessment

When a referral is received for a Nursing Home applicant located in a hospital, the SCDHHS worker must complete the assessment **within five (5) business days** of the case assignment date.

When a referral is received for a Nursing Home applicant located in a nursing home or community or a referral is received for a Community-Based applicant from any referral source, the **SCDHHS** worker must complete the assessment **within ten (10) business days** of the case assignment date.

Any exceptions to these timeframes must be documented in the narrative.

A level of care decision must be rendered **within three (3) business days** of the assessment being complete. Assessment completion may be delayed beyond the assessment date if additional information is needed from other knowledgeable sources. The SCDHHS worker will document any reason for delay in assessment completion in the narrative.

Note: Three (3) attempts, within a ten (10) business day timeframe, should be made to complete the assessment. These attempts must be on different days at different times. Each attempt must be narrated in Phoenix. After the third attempt, send out a CLTC Notification Form 171 to the applicant/Authorized Representative including the appeals notice. A copy of the Form 171 should also be sent to Medicaid Eligibility. If no response is received within ten (10) business days, then terminate the application.

02.40 Application Withdrawal

After intake, an applicant who does not appear to exhibit functional deficits can withdraw application prior to or during the assessment process. The SCDHHS worker will explain the level of care criteria and the reason why the applicant most likely does not meet the criteria. If the SCDHHS worker and the applicant determine that application withdrawal is appropriate, the CLTC Application Withdrawal Form, DHHS Form 150, must be reviewed with the applicant and/or Authorized Representative and the signature of the applicant and/or Authorized Representative can be requested. The SCDHHS worker should not utilize the Application Withdrawal Form if there are any questions about the applicant's ADL/Medical condition, or if the applicant/Authorized Representative does not agree with the withdrawal of the application. Once the applicant and/or Authorized Representative signs the form, the program application and case, where applicable, will be terminated in the Status Changes section of Phoenix as "Declined Participation." The SCDHHS worker will advise the applicant and/or Authorized Representative that a new application can be made at any time and will discuss possible community resources which may meet the applicant's needs. In the event the applicant and/or Authorized Representative declines to sign the form after it has been explained or disagrees with the SCDHHS worker's findings, the assessment process must continue using established policy and procedures.

02.50 Accurate Assessment Completion

The SCDHHS worker will contact the applicant and/or family, hospital, or agency, as appropriate, to arrange for assessment completion on assigned cases. The SCDHHS worker must document all attempts to schedule the assessment in the narrative record. The SCDHHS worker may complete the assessment by conducting a face-to-face visit, record review, or by a telephone interview. Phone assessment is not appropriate if information received is not clear, the information received is questionable, or the Level of Care is not clearly Intermediate or Skilled. In these situations, a face-to-face visit is required. A face-to-face visit should be made if the applicant has impaired ability to communicate due to hearing or speech impairment, language barrier or upon the applicant's request. The full Phoenix assessment will be completed when a face-to-face assessment visit is completed. Additional input from the physician or knowledgeable other(s) may be required.

The SCDHHS worker will obtain a signed Consent Form, DHHS Form 121, in Phoenix when a face-to-face assessment visit is made. If a telephone interview is conducted, the SCDHHS worker will obtain verbal consent. The verbal consent will be indicated on the Consent Form, DHHS Form 121 and must be signed by the person receiving the consent and one other witness. (A signed Consent Form, DHHS Form 121, in Phoenix will be obtained at the Initial Visit by the on-going Case Manager.)

The SCDHHS worker will obtain a signed Participant Service Choice Form, DHHS Form 164, in Phoenix when a face-to-face assessment visit is made. If a telephone interview is conducted, the SCDHHS worker will obtain a verbal choice. The verbal choice will be indicated on the Service Choice Form, DHHS Form 164, and must be signed by the person receiving the service choice. (A signed Service Choice Form, DHHS Form 164 in Phoenix will be obtained at the Initial Visit by the on-going Case Manager.) The Service Choice Form does not require a witness signature.

The assessment form must be completed accurately, obtaining all available information. Each applicable Phoenix section must be completed for both face-to-face and phone assessments. The assessor should read the South Carolina Assessment & Level of Care Manual for Medicaid-Sponsored Long-Term Care Services before completing an assessment and reference help sections in Phoenix. Only assessment information obtained and verified by the SCDHHS worker should be entered into Phoenix. Required assessment sections are determined by the type of program application. Each program designated section must be completed. It is the SCDHHS worker's responsibility to ensure that the assessment is properly completed before a level of care decision is determined.

When the assessor has created a phone assessment in Phoenix, and then it is determined that a face-to-face assessment is required, the assessor should complete a new assessment in Phoenix.

Accurate assessment completion requires the assessor to ask comprehensive and interview led questions. If accurate assessment information cannot be obtained during a phone assessment, a face-to-face assessment is required. The assessor must attempt to

involve the applicant as much as possible in the assessment process.

When an applicant is physically or mentally unable to participate in an interview, the guardian, or Authorized Representative can be interviewed for completion of the assessment. Assessors should complete the Mental Status Questionnaire (MSQ) to aid in the determination of the applicant's mental awareness.

The assigned SCDHHS worker informs the applicant and/or Authorized Representative of options concerning nursing home care or CLTC waiver services; obtains a Participant Service Choice Form for CLTC waiver services, DHHS Form 164 and Consent Form, DHHS Form 121; completes all appropriate sections/tabs in Phoenix; annotates the Narrative and Narrative Checklist; completes correspondence as appropriate to referring agency, applicant, and other individuals involved with the applicant; scans any forms and documents to the appropriate scan tag and routes correspondence to the designated Support Staff for mailing and/or filing as appropriate.

The designated Support Staff mails correspondence, and files as indicated.

The Medicaid category should be confirmed in Phoenix or in MMIS prior to proceeding with the assessment. All information in the applicant's information tab should be completed and confirmed (social security number, Medicaid ID, address, and phone number).

Note: Non-waiver supports, such as Home Health, should be verified by contacting the providing agency and verifying services provided to avoid duplication of services. This should include the service type/discipline, start of care date, and the frequency of service.

02.60 Medical Information Completion

The Diagnoses/Conditions section of the Assessment is the medical summary. This section must be completed on all applicants reviewed by SCDHHS for a final level of care determination. The assigned SCDHHS worker completes all appropriate information in Phoenix.

The SCDHHS worker should not delay the level of care decision in an effort to have diagnosis/conditions section verified by a medical source. However, if the SCDHHS worker is not able to identify reliable and accurate information, he/she should seek assistance from: physician, physician's staff, hospital staff, medical record, home health agency, Authorized Representative/applicant, and/or involved agencies. The SCDHHS worker is responsible for obtaining the most reliable and accurate information available.

02.61 Obtaining Medical Information for Medically Ineligible Applicants

The Physician Input Letter should be obtained for participants/applicants who have a skilled need or skilled service need without a complete functional deficit. The SCDHHS worker will seek medical information from the physician or primary care provider by mailing or faxing the Physician's Input Letter and the CLTC Consent Form 121. The Physician's Input Letter is available in Forms. The SCDHHS worker will add the source of the medical information in the Phoenix Source of Information/Level of Care section. The SCDHHS worker should consult with Central Office on difficult cases that cannot be resolved in the local SCDHHS Area Office or on cases where efforts to verify medical information from the physician or physician's staff have been unsuccessful. The applicant cannot be closed as Medically Ineligible until receiving input from the physician.

Three (3) attempts within ten (10) business days should be made with the physician's office to coordinate completion of the physician information form. If the form remains incomplete after three (3) attempts within ten (10) business days, then additional information can be obtained by phone from the physician's office and narrated in Phoenix. The assessment may be team discussed with the lead team nurse based on the information that was provided to the SCDHHS worker during the initial assessment.

If a waiver applicant appears to be medically ineligible, the SCDHHS worker must complete the following tasks:

- Obtain medical information from the physician by mailing or faxing the Physician's Input Letter and the CLTC Consent Form 121.
- Contact the applicant/Authorized Representative for clarification as necessary which may include a face- to-face visit by a Nurse Consultant.
- Conduct a team conference with another Nurse Consultant to reach a joint level of care decision.

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- If an applicant is Medically Ineligible after assessment and completion of the Physician Information Form, the nurse should send the case to the Area Administrator and/or the Lead Team Nurse for review prior to closing the case as Medically Ineligible. If the Level of Care remains in question, it should be sent to the Central Office Designee for further review.
 - If a level of care decision cannot be reached, consult with the Area Administrator. If efforts to verify medical information with the physician are unsuccessful, (i.e.: unable to obtain a completed MD Input Form) the case may be staffed with the Area Administrator based current information available. If the SCDHHS worker and the Area Office Administrator are unable to make a level of care determination, forward case to the Waiver Service Exception group for final level of care determination.
 - Complete the Level of Care section in Phoenix.

Note: The assessor should not sign the Source of Information/ Level of Care tab until all assessment information has been obtained. This marks assessment completion. The reviewer will sign and date following the level of care team conference unless the case will be sent for Central Office review. In these cases, the reviewer will not sign until a final level of care determination is made. Level of Care Exceptions made by Central Office require a third signature.

- Discuss Level of Care decision and possible alternatives with applicant and/or Authorized Representative.
- Create and send a CLTC Notification Form 171 to Central Medicaid Eligibility to notify them of the case closure.
- Send a Client Notification Form, including the appeals notice, to the applicant/Authorized Representative informing them of case closure. Applicant/Authorized Representative has ten (10) business days to respond from the date of the notification.
- Document all activities in the Narrative.
- Terminate the application effective date of case closure.

02.70 Pre-Admission Review for Waiver Enrollment

Pre-admission review is completed by a SCDHHS worker for an applicant who applies for waiver enrollment. Policy and procedure must be followed for intake, assessment, level of care, and Medicaid financial eligibility.

The pre-admission review information must provide accurate and comprehensive information. The Assessment must present a clear picture of the applicant's medical, skin/nutrition, functional abilities, and communication/psycho-behavioral status. In addition, the following information must be addressed and documented in the applicant's record by the SCDHHS worker:

- Setting during interview process, including persons present, applicant involvement in assessment process and explanation of CLTC program. If the assessment is completed by phone, indicate the individuals participating in the phone interview.
- Special alerts, including pets in the home, type of pets and if the pets are dangerous or bite, smoking in the home and oxygen in use, and reports of bedbugs in the home.

During face-to-face or phone assessments, the SCDHHS worker will verify the accuracy of all information in the Participant Information section.

- If the assessment is completed by home visit, a CLTC brochure will be provided informing the applicant/Authorized Representative of available waiver services. If the assessment is completed by phone, the SCDHHS worker will explain the available waiver services verbally.

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If the assessment occurs in the home, the SCDHHS worker completes the Home Assessment, Caregiver Supports and the Medication tab.

The SCDHHS Area Office and SCDHHS worker's contact information will be provided. Referrals should be made to other agencies for the immediate needs of the applicant as appropriate, i.e., Adult Protective Services. Any referrals made on the applicant's behalf will be documented in the narrative.

If the applicant does not enroll in the waiver **within thirty (30) calendar days** of the initial assessment, a new assessment must be completed. The SCDHHS worker must contact the applicant and/or Authorized Representative to confirm the information obtained in the assessment and document this contact in the narrative. (It may be necessary to re-verify Medicaid financial eligibility depending on the amount of time the case has been pending.) This contact must be made before the case is transferred to the **Case Manager II** for waiver entry and service plan development.

If the applicant appears to be medically ineligible at any time while the case is pending, a

face-to-face visit must be made to complete a new assessment and re-determine the level of care.

The date the applicant is determined to be eligible (level of care, location of care, Medicaid financially eligible, wants to participate in the waiver, and slot available) to enter the waiver must be documented in the narrative by the **SCDHHS** worker.

The SCDHHS worker must call to confirm the participant's location prior to the transfer conference and enrollment.

The SCDHHS worker and the Case Manager II must team staff the assessment information for service plan development and to enter the case into the waiver within five (5) business days of the applicant becoming eligible. Any delays in case transfer must be documented in the narrative.

The SCDHHS worker completes a narrative entry documenting the team conference with the Case Manager II was held.

Note: The applicant must be enrolled in the waiver by the Case Manager II on the same day of the SCDHHS worker/Case Manager conference.

The enrolling Case Manager II must complete the following tasks on the date of enrollment which is the same date of the transfer conference:

- Narrate the transfer conference with the SCDHHS worker
- Change the Participant Status to “participating” in Phoenix,
- Notifies the designated CLTC Support Staff for RSP indicator entry. The RSP entry must be completed within four (4) business days of the waiver enrollment date.

NOTE: Send a CLTC Notification Form to the participant, and/or Authorized Representative after contact with them to notify of the waiver enrollment.

02.80 Transfer of Pending Waiver Applicant to another CLTC Region

When a pending waiver applicant, who has been assessed by a SCDHHS worker, moves to another CLTC region, the transfer conference may be accomplished by telephone between the transferring region SCDHHS worker and the receiving region Case Manager II. This policy applies to an eligible applicant (level of care, location of care, Medicaid financially eligible, wants to participate and waiver slot available.)

Note: If dis-enrollment from a non-compatible waiver or managed care plan is necessary, this must be completed online, and the change reflected in Phoenix by the transferring SCDHHS worker prior to the transfer.

The SCDHHS worker will initiate the Phoenix transfer process and will remain assigned to the applicant in pending waiver status until the transfer telephone conference is completed with the receiving SCDHHS Area Office. The transferring SCDHHS worker will ensure all forms or documents not completed in Phoenix are scanned to the participant's Phoenix record. Once the transferring office cues Phoenix to transfer the case, the transferring CLTC Regional Office Case Management Team and Support Staff will no longer have access to the record. **Note:** Nurse Consultants have statewide Phoenix access. The transferring SCDHHS worker should complete all actions before cueing the Phoenix transfer. The **receiving Case Manager II** routes the information to the **Support Staff** who will mail any items to be sent to the participant and/or Authorized Representative.

The Designated Support Staff makes the appropriate copies as directed and routes to the designated persons/agencies.